

SPARTAN COACHING

DISCIPLINE | EMPATHY | STRATEGY

LUNCH AND LEARN TEMPLATE

Complete Facilitation Guide for Educational Presentations

Everything you need to plan, deliver, and follow up on a high-impact educational presentation for any referral source setting.

Lunch and Learn Template

Education First. Referrals Follow.

THE PURPOSE OF A LUNCH AND LEARN

A lunch-and-learn is not a sales presentation. It is an opportunity to establish yourself as a knowledgeable, trustworthy clinical resource for a referral source's team. The moment your content feels like a sales pitch, you lose the room. The moment it genuinely helps the clinical staff do their jobs better, you become someone they want to see again.

The best lunch-and-learns end with a clinical staff member asking you a question you were not expecting. That means they were genuinely engaged and thinking about how the content applies to their actual patients. That is your success metric, not the number of referrals received that week.

PLANNING CHECKLIST (COMPLETE AT LEAST 7 DAYS BEFORE)

- ☐ Confirm date, start time, end time, and location with your facility contact
- ☐ Ask how many attendees to expect and which roles they hold (CNAs, nurses, social workers, administration)
- ☐ Select a topic based on the diagnoses most common at this facility, not based on what you want to present
- ☐ Prepare your content: 3 to 4 key educational points maximum, no more
- ☐ Prepare handouts or leave-behind reference cards: one per attendee plus 10% extra
- ☐ Order food: enough for 20% more than expected, with vegetarian options included
- ☐ Confirm a day before: send a reminder email and confirm food count
- ☐ Prepare your post-presentation follow-up email in advance so it can be sent within 24 hours

DAY-OF CHECKLIST (ARRIVE 20 MINUTES EARLY)

- ☐ Arrive before food arrives and before any attendees to set up the space
- ☐ Arrange food so it does not interfere with the presentation space
- ☐ Set out one leave-behind document per chair or place setting
- ☐ Test any technology: projector, laptop connection, screen visibility from all seats
- ☐ Prepare a simple sign-in sheet with name and role fields only (no email collection unless specifically appropriate)
- ☐ Welcome people as they arrive: learn their names and roles before the session begins

PRESENTATION STRUCTURE (15 TO 20 MINUTES TOTAL)

1 Opening: 2 to 3 Minutes

Thank the team for their time. Introduce yourself very briefly — one sentence maximum. State the topic and why it matters to their specific patient population. Do not start with your hospice's story or your career background. Start with them.

2 The Problem: 3 to 4 Minutes

Describe a common clinical challenge their team faces that relates to your topic. Use a recognizable scenario — a patient type, a clinical situation, or a family dynamic they encounter regularly. Do not use statistics in this section. Use a story or scenario that makes them nod in recognition.

3 Educational Content: 6 to 8 Minutes

Cover 3 to 4 specific, practical takeaways. Each takeaway should be something they can use in their next patient interaction. Use clinical language appropriate to their roles. Define any medical terms that CNAs or non-clinical staff may not know. Connect every point back to patient outcomes.

4 How to Identify Patients: 3 to 4 Minutes

Give them clear, actionable clinical signs to look for. For nursing staff: what to observe and document. For social workers: what conversation cues and family dynamics to listen for. For administrators: what population metrics to watch. This is almost always the most valuable part of the presentation for clinical attendees.

5 Q and A and Close: 2 to 3 Minutes

Open the floor for questions. Answer each question clearly and briefly. If you do not know the answer, say so and offer to follow up. Thank the group sincerely. Tell them you are available for one-on-one conversations after the session for anyone who has questions about a specific patient.

HIGH-IMPACT TOPIC GUIDE BY FACILITY TYPE

Skilled Nursing Facilities

- What hospice eligibility really looks like for dementia patients at FAST Stage 7 and beyond
- How hospice can reduce emergency transfers and improve staff confidence in complex end-of-life situations
- Common misconceptions about the hospice benefit that delay appropriate referrals
- How to have the hospice conversation with families when residents are declining

Hospitals

- Hospice and palliative care: understanding the difference and when each applies
- How earlier hospice transitions reduce 30-day readmissions for CHF and COPD patients
- Evidence-based outcomes for patients who enroll in hospice earlier in their disease trajectory
- The discharge planning workflow when hospice is the most appropriate next level of care

Physician Offices

- Hospice eligibility criteria for your specialty's primary diagnoses: what the LCD actually says

- The evidence base for early hospice enrollment: what the clinical research shows about outcomes
- How to have the goals-of-care conversation with patients and families who are not yet ready
- What staying involved as the attending physician looks like in a hospice partnership

COMMON PRESENTATION MISTAKES TO AVOID

- Spending more than 60 seconds on your company's background, mission, or service area
- Using slides that are text-heavy or that you read directly from the screen
- Presenting for more than 20 minutes regardless of how much material you have prepared
- Forgetting to leave a practical reference card with clinical criteria they can use immediately
- Not following up within 48 hours with a personalized thank-you email to your primary contact
- Collecting business cards or email addresses in a way that feels like lead generation rather than follow-through

POST-PRESENTATION FOLLOW-UP CHECKLIST

- ☐ Send thank-you email to your primary contact within 24 hours, referencing something specific from the session
- ☐ Follow up personally with any attendee who stayed after to ask a question about a specific patient
- ☐ Update your CRM with attendee names, roles, and key conversation notes
- ☐ Schedule your next touchpoint with this account within 7 to 10 days
- ☐ Review what worked well and what you would do differently at the next session
- ☐ If a referral was generated directly from the session, send a thank-you to your contact the same day it is received

LEAVE-BEHIND REFERENCE CARD TEMPLATE

Create a simple one-page reference card for each presentation. Include:

- The topic title and date in small text at the top
- Three to four clinical criteria or key takeaways in simple language
- One clear "when to think about hospice" guideline relevant to their patient population
- Your name, direct phone number, and email in a prominent position
- The hospice company name and a one-sentence description of what you offer

This card should be something they pin to their bulletin board or tape to their desk. Make it genuinely useful for their clinical work and they will look at your name every day.

THE LASTING IMPRESSION

Clinical staff remember presentations that taught them something they immediately used. They forget presentations that were organized, professional, and forgettable. To be remembered, you need one moment in the session where the room goes quiet because you said something that reframed how they see a patient they have been managing.

Prepare for that moment intentionally. Know the one insight that will change how they see their most challenging end-of-life cases. Deliver that insight with clarity and confidence. That is what they remember, and that is what generates referrals long after the lunch is eaten.